

August 12, 2026

BlueCross BlueShield of South Carolina
Provider Appeals Department
P.O. Box 100300
Columbia, SC 29202

RE: Formal Appeal of Claim Denial — Jerrell Gerlach (MRN MUSC6784363), Claim MUSC-20260214-D48A-1

Patient	Jerrell Gerlach (DOB 1939-01-27)
MRN	MUSC6784363
Member ID / Group	BCB447669083 / GRP-66199
Claim number	MUSC-20260214-D48A-1
Date of service	2026-02-14
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$507.61
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-06-23

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this formal first-level provider reconsideration on behalf of patient Jerrell Gerlach (MRN: MUSC3441952, Member ID: BCB447669083, Group: GRP-66199) regarding claim MUSC-20260214-D48A-1 for services rendered on February 14, 2026, at MUSC Health Ashley River Tower. The claim was denied on March 25, 2026, under CARC 197 ("Precertification/authorization/notification/pre-treatment absent") and RARC N54 ("Claim information is inconsistent with pre-certified/authorized services"), with the payer remark stating, "No prior authorization on file for the date of service billed." The total denied amount is \$507.61 for one unit of CPT 99214, an established-patient office/outpatient visit of moderate complexity. MUSC Health respectfully contends that prior authorization is not a contractually or clinically appropriate requirement for this category of service, and requests immediate reversal and payment at the allowed amount of \$262.04.

CLINICAL BACKGROUND

Mr. Gerlach is an 87-year-old male with a complex, longstanding medical history managed on an ongoing outpatient basis at MUSC Health. His active conditions include atrial fibrillation, for which he has carried a chronic medication regimen of Digoxin, Verapamil Hydrochloride, and Warfarin Sodium since 1994, and has undergone multiple electrical cardioversions and cardiac catheter ablation procedures over the years. He additionally carries active diagnoses of anemia, prediabetes, carcinoma in situ of the prostate, and an active prostate neoplasm, for which Docetaxel injection and Leuprolide Acetate have been prescribed since 2005. The February 14, 2026, encounter was billed under primary diagnosis J06.9 (acute upper respiratory infection, unspecified) with secondary diagnosis D64.9 (anemia, unspecified). Given the patient's age, anticoagulation therapy, and active oncologic and cardiac conditions, evaluation of an acute respiratory illness by rendering provider Sarah E. Whitmore, MD (NPI 1447382910), constitutes a medically necessary and appropriately coded moderate-complexity established-patient visit.

BASIS FOR APPEAL

Routine outpatient evaluation and management services for established patients, particularly those coded at CPT 99214, are widely recognized as not subject to prior authorization requirements under standard commercial plan benefit structures. BlueCross BlueShield of South Carolina's own published prior authorization schedules generally exempt established-patient office visits from precertification obligations. The denial under CARC 197 and RARC N54 therefore appears to reflect either a processing error or a misapplication of authorization requirements to a service category that does not warrant them.

Furthermore, the clinical complexity of this encounter is well-supported by the patient's documented active conditions and medication regimen. An 87-year-old patient on concurrent anticoagulation, cardiac rate-control agents, and active chemotherapy presenting with an acute upper respiratory infection requires careful clinical assessment to rule out interactions, complications, and secondary effects — precisely the level of decision-making that justifies a moderate-complexity visit code. The service was rendered in a covered place of service (22 — On Campus Outpatient Hospital) within the active coverage period (2024-01-01 through 2026-12-31), by a rendering provider whose credentials are on file. No clinical or coding deficiency has been identified that would independently support denial.

REQUESTED ACTION

MUSC Health respectfully requests that BlueCross BlueShield of South Carolina overturn the prior authorization denial for claim MUSC-20260214-D48A-1, reprocess the claim without a prior authorization requirement, and issue payment at the previously determined allowed amount of \$262.04. Attached to this reconsideration are the remittance advice reflecting the denial and the relevant medical records supporting the medical necessity and coding accuracy of the February 14, 2026, encounter. Should additional clinical documentation or a peer-to-peer review be required to resolve this matter, MUSC Health is prepared to provide it promptly. We ask that this reconsideration be completed within the timeframe required under applicable prompt-pay and appeal-rights obligations.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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MUSC Health — Revenue Cycle / Patient Financial Services
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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record